

BIOSYNC CLINICAL LABORATORY REPORT — URGENT

Patient: Test Account (DOB: 01/01/1990) | Date: April 28, 2026

Vital Signs & Biometric Readings

Heart Rate	161 BPM
Blood Pressure	191/128 mmHg
SpO2	84%
Temperature	104.1°F
Glucose	341 mg/dL
Weight	172 lbs
BMI	24.1
HRV	11 ms

Clinical Notes

Patient brought in via ambulance — unresponsive at home. No known medications. No known allergies. No prior surgeries. Previously healthy per April 1 wellness visit. Heart rate critically elevated at 161 BPM — life-threatening tachycardia. IV adenosine administered. Blood pressure 191/128 mmHg — hypertensive emergency, IV nitroprusside initiated immediately. SpO2 critically low at 84% — patient intubated on arrival, mechanical ventilation in progress. Temperature 104.1°F — critical hyperpyrexia, active cooling measures applied, sepsis workup initiated. Glucose 341 mg/dL critically elevated — no prior diabetes diagnosis, insulin drip started, HbA1c ordered. HRV 11 ms — severely depressed, consistent with multi-system failure. Patient unresponsive on arrival — GCS score 5. Neurology emergency consult placed. CT head ordered. All body systems in acute crisis simultaneously.

Provider Notes

Patient in multi-organ crisis — full ICU admission. Cardiology: IV antiarrhythmics and antihypertensives running, continuous cardiac monitoring, defibrillator on standby. Pulmonology: mechanical ventilation, PEEP titrated, repeat ABG every 2 hours. Endocrinology: insulin drip for hyperglycemic crisis, electrolytes corrected. Infectious disease: broad spectrum antibiotics initiated — septic source unknown, blood cultures x3 drawn, urine and sputum cultures sent. Neurology: CT head showing no acute hemorrhage — hypertensive encephalopathy suspected. All specialists at bedside. Emergency contact notified. Prognosis critical. Ordering Provider: Dr. James Henderson, MD — URGENT. Facility: BioSync Clinical Health Center — ICU